

Blood Pressure - Low - Male (Adult)

Algorithm 1409 | Adult (18 years and older) | Gender at birth: Male

Definition

- UAT assessment structure for a low blood-pressure reading or symptoms compatible with hypotension. A reading is interpreted with age, pregnancy/postpartum status, symptoms, trend, measurement quality, prescribed baseline, and possible causes such as bleeding, severe infection/sepsis, anaphylaxis, cardiac illness, dehydration, heat illness, or medication effects. A single home-device value does not diagnose shock and a reassuring value must not override serious symptoms.
- Adult patient.
- Male.

Initial assessment

1. [OPEN_TEXT] What is the person's exact age and sex, and are they pregnant or within 6 weeks after birth? For a child, record the exact age rather than applying an adult blood-pressure threshold.
2. [OPEN_TEXT] What were the blood-pressure readings, when were they taken, and what is the person's usual or clinician-set range? Was the correct cuff size used on a supported bare arm at heart level after rest, and was the reading repeated?
3. [YES_NO] Is the person unresponsive, difficult to wake, confused, fainting, unable to stand, very weak, pale/mottled/cold/clammy, breathing abnormally, or passing very little or no urine?
4. [YES_NO] Is there chest pain, severe breathlessness, a very fast/slow/irregular heartbeat, stroke signs, seizure, severe abdominal or back pain, major injury, or visible or suspected internal bleeding such as vomiting blood, black/bloody stool, or heavy vaginal bleeding?
5. [YES_NO] Is there possible severe infection or sepsis: rapid deterioration, fever or unusually low temperature, shaking, abnormal colour, severe pain, fast/difficult breathing, confusion, unusual sleepiness, poor feeding, repeated vomiting, or reduced urine/wet nappies?
6. [YES_NO] After a possible allergen, is there throat/tongue swelling, wheeze or breathing difficulty, widespread hives, vomiting, sudden dizziness/faintness, confusion, floppiness, or pale/cold/clammy skin?
7. [YES_NO] Could dehydration, heat exposure, vomiting/diarrhoea, poor intake, a new or changed medicine, overdose, pregnancy complication, or a heart/endocrine condition be contributing?

Triage and redirect questions

1. Redirect → Fainting

Was there a brief loss of consciousness with recovery, rather than ongoing low blood pressure symptoms?

A brief loss of consciousness belongs in the Fainting pathway.

1. Disposition level 100

Is the person unresponsive or difficult to wake; repeatedly fainting; confused; unable to stand; severely weak; pale, mottled, cold or clammy; breathing abnormally; or rapidly worsening? Is there chest pain, severe breathlessness, a dangerous-feeling heartbeat, stroke signs, seizure, major trauma, severe abdominal/back pain, heavy or suspected internal bleeding, severe infection/sepsis features, or suspected anaphylaxis? In a baby or child, include floppiness, abnormal interaction, poor feeding, abnormal breathing, abnormal colour, repeated vomiting, or markedly reduced urine/wet nappies. During pregnancy or within 6 weeks after birth, include faintness with bleeding, severe abdominal pain, chest/breathing symptoms, severe headache/visual symptoms, or collapse.

Hypotension with impaired consciousness, breathing/circulation abnormality, bleeding, anaphylaxis, cardiac or neurological symptoms, sepsis, trauma, or pregnancy/postpartum danger signs may represent shock or another time-critical condition. Emergency disposition is driven by the whole clinical picture, not a numeric threshold or one device reading.

1. Disposition level 70

With all emergency features absent, is this a child with a reported low reading or new dizziness/weakness; a pregnant or recently postpartum person with a new low reading or symptoms; or anyone with persistent/recurrent dizziness, near-fainting, blurred vision, nausea, weakness, postural symptoms, reduced intake/urine, vomiting/diarrhoea, heat exposure, or a new/changed medicine?

Persistent or recurrent symptoms need prompt assessment for dehydration, medication effects, pregnancy-related causes, cardiac/endocrine illness, or evolving infection. Pediatric blood pressure is age- and size-dependent, so this UAT protocol does not apply an adult cutoff to a child.

1. Disposition level 40

Are there no symptoms or danger features, with only one unexpectedly low home/device reading or a reading consistent with the person's clinician-documented usual range?

One isolated reading can be affected by cuff size, position, rest, movement, device limitations, or the person's normal baseline. A properly repeated measurement and clinician review are safer than diagnosing or treating from a single number; Qatar service route and timing require local approval.

Advice

Qatar emergency response for shock or a dangerous underlying cause

- Call Qatar emergency services on 999 now and follow the call-taker's instructions; do not drive or let the person walk. Keep them still and warm. If faint but breathing normally, lay them flat if tolerated; use the recovery position if unconscious but breathing. If late in pregnancy, place them on their left side unless breathing is easier in another position or 999 advises otherwise. If breathing is difficult, allow the position that makes breathing easiest. Control visible external bleeding with firm direct pressure if safe. If anaphylaxis is suspected, use the person's prescribed adrenaline auto-injector immediately and follow their emergency plan. If unresponsive and not breathing normally, start age-appropriate CPR as directed by 999 and use an AED if available. Do not give food, drink, salt, caffeine, or unprescribed medicine, and do not delay for another blood-pressure reading. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: reduced consciousness or repeated fainting; abnormal breathing, chest pain, or stroke signs; cold, clammy, pale, mottled, or rapidly worsening condition; bleeding or major trauma; sepsis or anaphylaxis features; pregnancy or postpartum danger signs.

Prompt in-person assessment for symptomatic or higher-risk low blood pressure

- Arrange prompt same-day in-person assessment through the Qatar route approved by local clinical governance. Keep the person seated or lying until dizziness settles, assist them when standing, and prevent falls. Small sips of fluid may be reasonable only if fully alert, not vomiting, and without a clinician-imposed fluid

restriction; do not force fluids in a baby, pregnant person, or anyone with heart/kidney disease. Do not stop, skip, or change prescribed medicine without clinician advice. Record readings, symptoms, timing, recent illness/intake, pregnancy/postpartum status, and medicines. Call 999 if any emergency feature appears or the person cannot be safely transported.

- Call Back If: fainting, confusion, or inability to stand; chest pain or abnormal breathing; bleeding, severe pain, or pregnancy/postpartum concern; fever, abnormal colour, reduced urine, or rapid deterioration; allergic swelling or wheeze.

Repeat and review an isolated asymptomatic reading

- If the person remains completely well, rest quietly for at least 5 minutes, use a validated device and correctly sized cuff on a supported bare arm at heart level, avoid talking or moving, and repeat the measurement according to the device instructions. Record both readings rather than repeatedly checking. Arrange review through the Qatar non-emergency route and timing approved by local governance, especially for a child, pregnancy/postpartum patient, new medicine, or a result outside the person's clinician-documented usual range. Do not use this branch when symptoms or danger signs are present, and do not change medicine, deliberately increase salt, or advise extra fluid without considering pregnancy and heart/kidney restrictions.
- Call Back If: dizziness, weakness, fainting, or confusion; abnormal breathing, chest pain, bleeding, fever, or reduced urine; new concern in a child or pregnancy/postpartum; repeat reading remains unexpectedly different from documented baseline.

First aid

- Call Qatar emergency services on 999 now and follow the call-taker's instructions; do not drive or let the person walk. Keep them still and warm. If faint but breathing normally, lay them flat if tolerated; use the recovery position if unconscious but breathing. If late in pregnancy, place them on their left side unless breathing is easier in another position or 999 advises otherwise. If breathing is difficult, allow the position that makes breathing easiest. Control visible external bleeding with firm direct pressure if safe. If anaphylaxis is suspected, use the person's prescribed adrenaline auto-injector immediately and follow their emergency plan. If unresponsive and not breathing normally, start age-appropriate CPR as directed by 999 and use an AED if available. Do not give food, drink, salt, caffeine, or unprescribed medicine, and do not delay for another blood-pressure reading. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- Hamad Medical Corporation, "Life-Threatening Medical Emergency", <https://hamad.qa/EN/Emergency/Pages/Life-ThreateningMedicalEmergency.aspx> (accessed 2026-07-25)
- NHS.UK, "Low blood pressure (hypotension)", <https://www.nhs.uk/conditions/low-blood-pressure-hypotension/> (page last reviewed 11 July 2023)
- NHS.UK, "Blood pressure test", <https://www.nhs.uk/tests-and-treatments/blood-pressure-test/> (accessed 2026-07-25)
- NHS.UK, "Sepsis", <https://www.nhs.uk/conditions/sepsis/> (accessed 2026-07-25)
- NHS.UK, "Anaphylaxis", <https://www.nhs.uk/conditions/anaphylaxis/> (accessed 2026-07-25)

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

Manifest hash: 888885496ad13bc6983acf5812f442bc7acaa88c35c7b1bf1ab7695f061dd65b | Status: BLOCKED_BY_RESEARCH_GAP